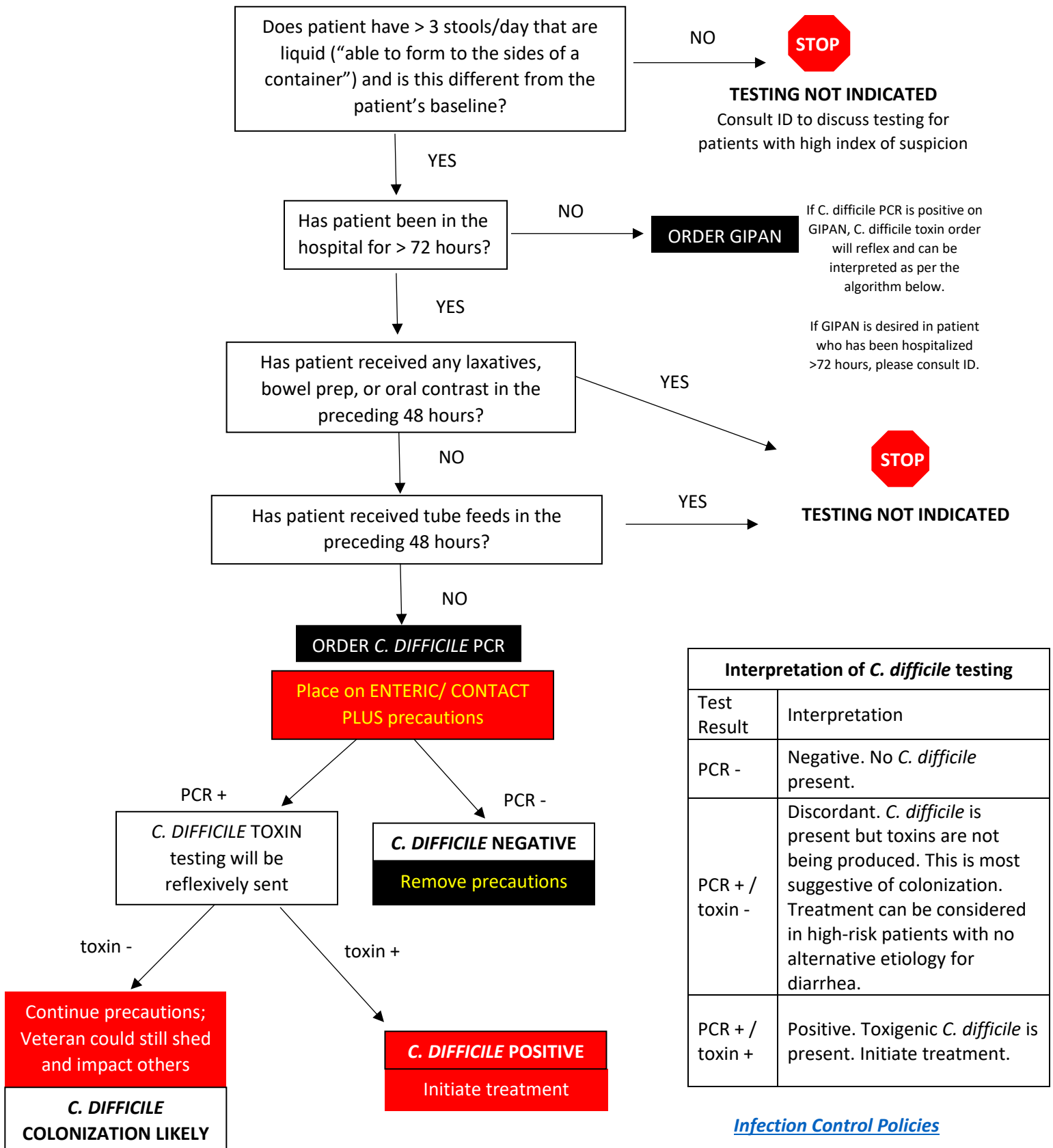


INFECTIOUS DIARRHEA TESTING ALGORITHM



CLOSTRIDIUM DIFFICILE INFECTION (CDI) TREATMENT GUIDANCE

- For all cases of CDI, please review systemic antibiotics and discontinue if not indicated
- Avoid PPI/H2 blockers without an appropriate indication
- Patients with a history of CDI should avoid systemic antibiotics unless clearly indicated to prevent recurrence

Initial Episode	
Non-severe (WBC <15K, SCr <1.5 mg/dL) OR Severe (WBC >15K, SCr >1.5 mg/dL or ≥50% rise from baseline SCr level)	Vancomycin PO 125 mg QID x 10 days Alternative: Fidaxomicin PO 200 mg BID x 10 days ¹
Fulminant (Severe + hypotension/shock/ileus/megacolon)	• Provide aggressive volume resuscitation • Consult Surgery and ID Vancomycin PO 500 mg QID x 48-72 hours or until clinical improvement THEN vancomycin PO 125 mg QID for an additional 10 days Consider addition of: Metronidazole IV 500 mg TID Vancomycin PR 500 mg QID (if ileus)
Recurrent CDI (rCDI)²	
First recurrence ² (≤ 90 days from prior episode)	Vancomycin PO 125 mg QID x 14 days then Vancomycin PO 125 mg BID x 7 days then Vancomycin PO 125 mg daily x 7 days then Vancomycin PO 125 mg every other days x 7 days then Vancomycin PO 125 mg every 3 days for 2-8 weeks Alternative: Fidaxomicin PO 200 mg BID x 5 days then PO 200 mg every other day x 20 days
Second or subsequent recurrence ² (≤ 90 days from prior episode)	• In coordination with an ID consult: Repeat prolonged taper as above then consider FMT³ after successful treatment of CDI FMT via capsule therapy (VOWST) >>> enema (REBYOTA) ➤ If FMT failure (i.e., recurrence of CDI within 8 weeks of treatment): Restart anti-CDI antibiotics (e.g., fidaxomicin or vancomycin) <i>before</i> repeat FMT

Notes:

1. **Fidaxomicin** requires ID approval and ID pharmacist evaluation of national VA criteria for use. Both fidaxomicin and vancomycin are appropriate therapies for treatment of recurrent C difficile per the Infectious Diseases Society of America (IDSA). Place an ID consult (e-consult for outpatient, ID consult for inpatient) for consideration of best option for individual patients. Fidaxomicin may be preferred for patients with increased risk of CDI recurrence (e.g. age >65 years old, concomitant antibiotic use during CDI treatment, and/or immunosuppression), however local rates of recurrence are much lower than reported in the vancomycin vs. fidaxomicin trials.

2. **Recurrent CDI and/or treatment failure:** Symptoms of CDI after 90 days from a prior episode is considered a new infection and can be treated as an initial episode. Failure is defined as no improvement or worsening symptoms after 48-96 hours of primary treatment. In those patients who do not improve, please consider alternative diagnoses (e.g., post-infectious IBS) or conversely progression to fulminant disease based on clinical status. Changes in therapy can be considered with ID guidance; however, note that there is no data to support switching treatment (e.g., from PO vancomycin to fidaxomicin). In veterans who fail to respond to standard therapy and who do not have an alternative etiology for diarrhea, FMT may be considered in consultation with ID.

3. **Fecal microbiota transplantation (FMT) products** require ID consultation and ID pharmacist evaluation of national VA criteria for use. Vowst™ (fecal microbiota spores in capsule) must be given 2-4 days after completion of antibiotics and is contraindicated for patients with ANC < 500 cells/mm³, those who will likely be treated with any antibiotics in the next 8 weeks (including a single pre-operative dose), those who cannot undergo the necessary pre-treatment with magnesium citrate or polyethylene glycol, and those cannot comply with administration on an empty stomach prior to the first meal of the day).

Miscellaneous:

- Consider preference of antibiotics that do not increase CDI risk in high-risk patients. Tetracyclines such as doxycycline are considered lower risk for CDI.
- Recommending a diet containing probiotics may be considered to reduce CDI risk, though data are mixed.
- Data are insufficient to recommend extending treatment courses for patients who remain on systemic antibiotic therapy after completion of CDI treatment but may be considered in discussion with ID.
- Cholestyramine binds to oral vancomycin and concomitant use should be avoided.

Last revised: 12/2025		Antimicrobial Stewardship Committee: 12/2025	P&T: 1/2026 ECMS: Pending
References			
	Citation		
1	McDonald CL, Gerding DN, Johnson S, et al. Clinical Practice Guidelines for Clostridium difficile Infection in Adults and Children: 2017 Update by the Infectious Diseases Society of America (IDSA) and Society for Healthcare Epidemiology of America (SHEA), Clinical Infectious Diseases, Volume 66, Issue 7, 1 April 2018, Pages e1–e48, https://doi.org/10.1093/cid/cix1085		
2	Johnson S, Lavergne V, Skinner AM, et al. Clinical Practice Guideline by the Infectious Diseases Society of America (IDSA) and Society for Healthcare Epidemiology of America (SHEA): 2021 Focused Update Guidelines on Management of Clostridioides difficile Infection in Adults. Clin Infect Dis. 2021;73(5):e1029-e1044. doi:10.1093/cid/ciab549		
3	Kelly CR, Fischer M, Allegretti JR, et al. ACG Clinical Guidelines: Prevention, Diagnosis, and Treatment of Clostridioides difficile Infections. Am J Gastroenterol. 2021;116(6):1124-1147. doi:10.14309/ajg.0000000000001278		
Revision History			
Date	Revision		